

TriageFlow pre-assessment rule flow

Proposed study procedure for nurse review | Adult fictional cases | Patient reports only

NO VITAL SIGNS OR PROFESSIONAL ASSESSMENT INPUTS

Rules must work on permitted patient reports. Clinical definitions and approval remain pending. The flow below describes the study design; its fact-to-rule mapping is not yet implemented.

1 Collect permitted patient-reported information

Age, complaint, symptoms, history, onset, duration, severity and worsening; reported tests or devices as context. Exclude measured vital signs, examination findings and laboratory or imaging result values.

METHOD A

Keep Luna's original priority or Needs Review result.

2 Save one shared Luna response

Preserve the extracted facts, evidence and states, plus the initial model priority. Both methods use the same permitted case information.

Extracted facts only enter Method B

3 Method B evaluates approved rules

Apply explicit conditions supported by permitted patient reports. Each rule records its source, required fields, allowed action, review approval and version. Luna does not choose Method B's priority.

4 Is a defensible rule decision supported?

An approved rule applies and its required facts support a consistent decision. Missing optional facts do not block it.

Yes

5 Select the supported preliminary priority

Where multiple approved rules apply, select the highest supported urgency (the lowest ESI number). A lower-priority rule cannot override it.

Save Method B result and rule trace

Provisional ESI-based level only when supported; otherwise Needs Review. Retain rule IDs, evidence and version beside Method A.

WHAT THE NURSE NEEDS TO DEFINE

For each proposed rule:

1. Permitted patient facts
2. Exact decision condition
3. Supported priority
4. When review is required

Needs Review

No supported rule, or necessary facts are unresolved. No priority assigned.

ESI v4 limitation

Standard Decision D uses vital signs. It cannot be completed inside this study. Skipping D does not establish ESI 3.

The nurse must establish which proposed levels can be supported from permitted facts. Do not invent substitutes for excluded inputs.

Study boundary: preliminary prioritization before assessment. Needs Review is a processing status. Tests already requested/completed and device presence do not automatically determine resources or priority. No new symptom thresholds are specified here.

Study source: thesis sections 1.5, 3.4-3.5 and 3.10-3.11 (supplied revision). Clinical structure reference: [AHRQ ESI v4](#). This is a proposed study flow, not an official ESI chart or a clinically validated rule set.